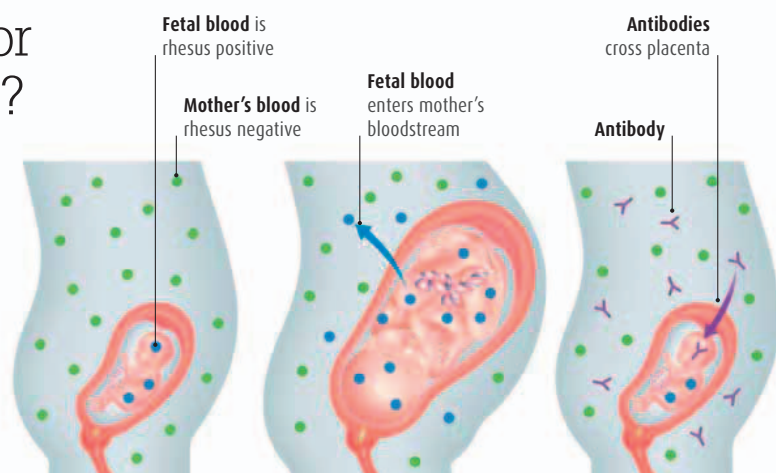


Q Why am I being tested for rhesus status? What is it?

In addition to having a blood type (A, B, AB, and O), all blood is either rhesus positive or rhesus negative.

A blood test early in pregnancy checks rhesus status. About 15 percent of people have rhesus negative blood. If your blood is rhesus negative and your baby's blood is rhesus positive, you may develop antibodies (defensive cells) if the baby's blood leaks into your blood. This contact can happen during a miscarriage or termination, at birth, or if you fall on your abdomen. It doesn't usually affect first babies, but once you develop antibodies, they can attack a subsequent rhesus-positive baby, causing anemia. In a first pregnancy, you'll have an Rh immunoglobulin (RhIg) injection to prevent antibodies from developing if you're rhesus negative. If your blood already has antibodies, extra ultrasounds will monitor the baby, and treatment may be needed.



1 In a first pregnancy, a rhesus-positive baby's blood will not cause issues unless it leaks into the rhesus-negative mother's body.

2 During childbirth the mother's rhesus negative blood creates antibodies in reaction to the "foreign" blood.

3 In a subsequent pregnancy, the antibodies can destroy a rhesus positive baby's blood cells, causing anemia in utero and jaundice after birth.

Q How will my doctor know if my baby is growing properly?

Your baby's growth is usually monitored by the growth of your uterus from its highest point (fundus) to the top of the pubic bone (symphysis pubis). This is called the "symphysis fundal height" (SFH) and, interestingly, the measurement in centimeters often corresponds to your baby's gestational age in weeks. Your doctor will check your fundal height at every prenatal appointment from 24 weeks until birth, using a

tape measure placed over your belly as you lie on your back. The fundal height is recorded on a percentile chart that plots this measurement against your baby's gestational age. At your next appointment a new fundal height measurement will indicate whether or not your baby is growing as expected. Before you are 24 weeks pregnant, your first ultrasound (see p.95) indicates how your baby is growing, and your doctor might listen to your baby's heartbeat at your checkups. Toward the end of pregnancy, your doctor will palpate your abdomen (see opposite) to check your baby's position as well as growth.

Q Do I need to go to childbirth classes? How do I choose them if I do?

Whether or not you go to classes is entirely up to you. However, there are great benefits if you do—including learning about your pregnancy, labor, and birth, and how to take care of your newborn baby. You will also meet other moms- and dads-to-be in your area due at the same time, which can be a great support when you are a new parent.

Your doctor will give you details of local childbirth classes—which may be run by labor and delivery nurses or midwives who work in your area. You can take more than one class, if you'd like to. Most childbirth classes begin at around eight to 10 weeks before the baby is due (at 30 to 32 weeks pregnant). If you are expecting twins or more, you should book your classes to begin when you are around 24 weeks pregnant, because your babies are more likely to be born early. Classes usually run for between three and six weeks, with one class every week, and each class lasting up to two hours.

You might find a six- or eight-hour class on a weekend for busy couples who don't have time to attend shorter evening classes over the course of several weeks.

Childbirth classes You are encouraged to bring along your birth partner, so that he or she can learn some specific techniques to relax you during labor.

